



NATIONAL INSTITUTE ON AGING · SBIR COMMERCIALIZATION READINESS PILOT

CareNavigator

Concepts for the CRP Application

PROJECT PERIOD

June 1, 2027 to May 31, 2030

THIS SET

40 numbered slides across the full application

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Research Strategy

Significance, Innovation, Approach, and the CRP Progress Report.

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Significance

Slides 3 to 12

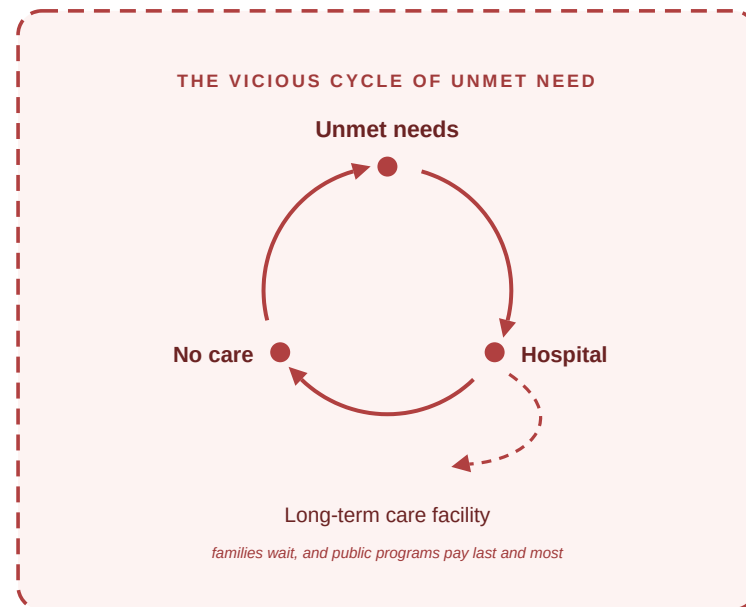
THE RESEARCH STRATEGY

Significance	Innovation	Approach	CRP Progress Report
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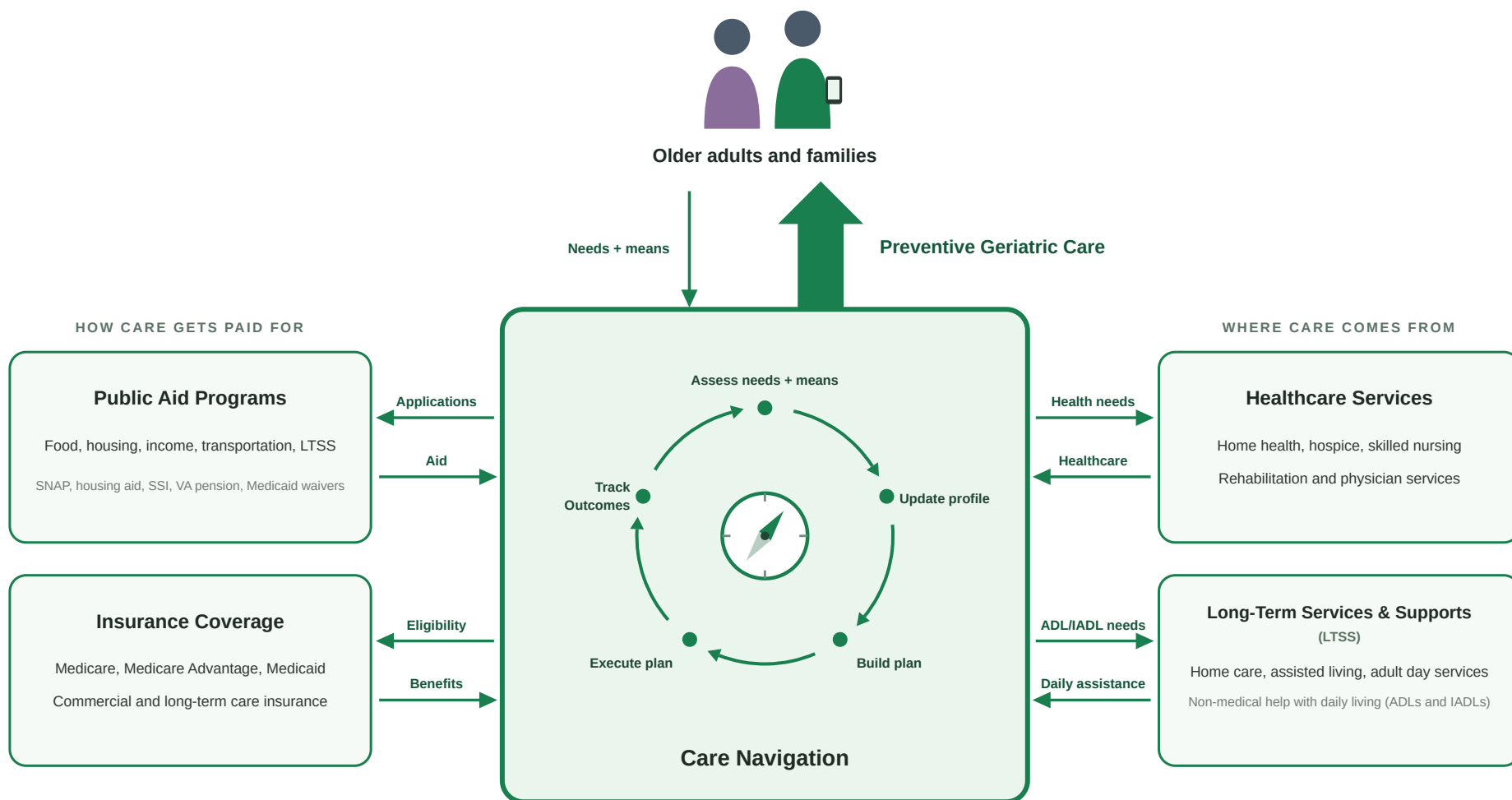
3. When Older Adults' Needs Go Unmet, a Vicious Cycle Follows



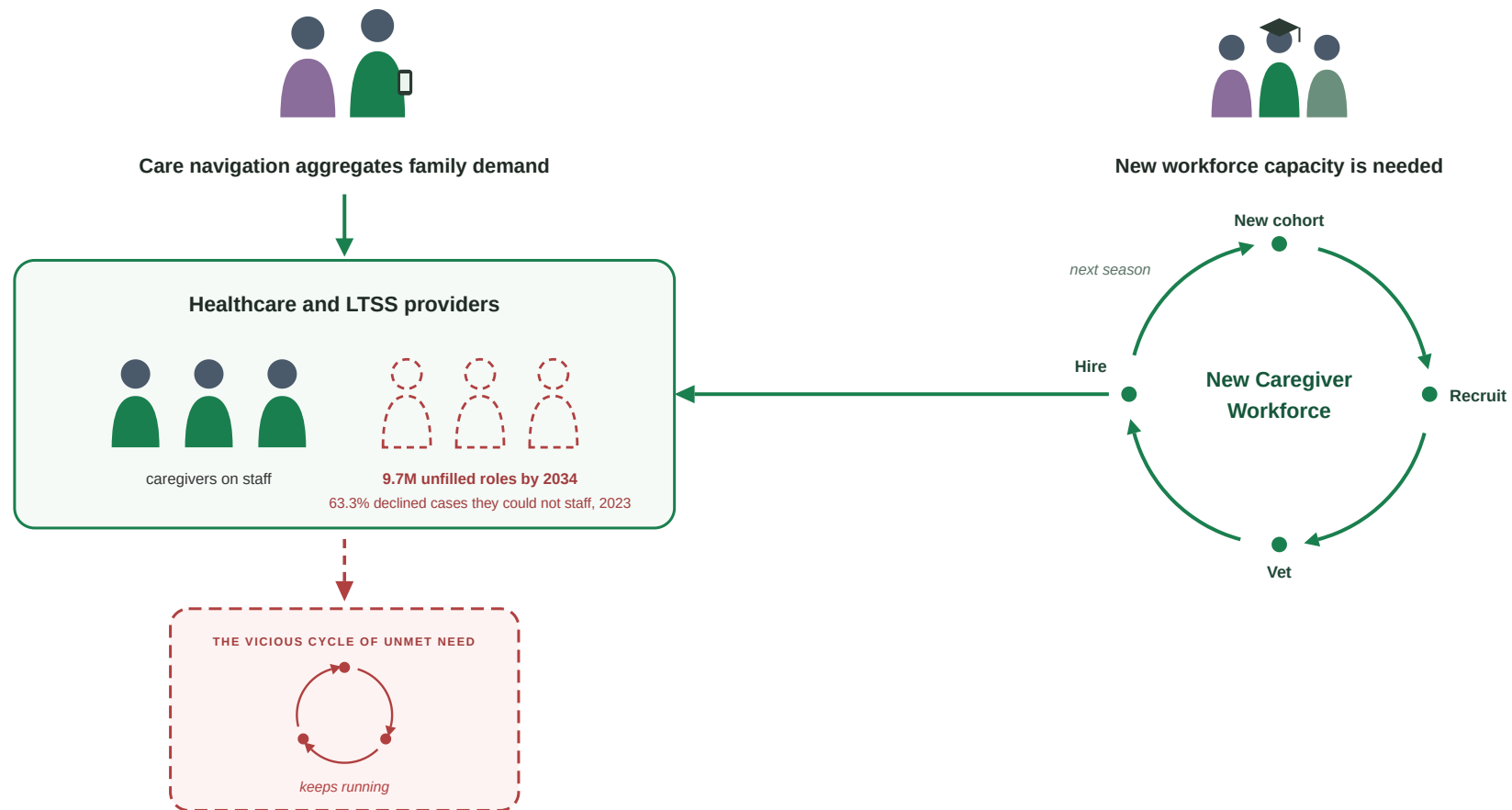
*too expensive to afford,
too confusing to find*



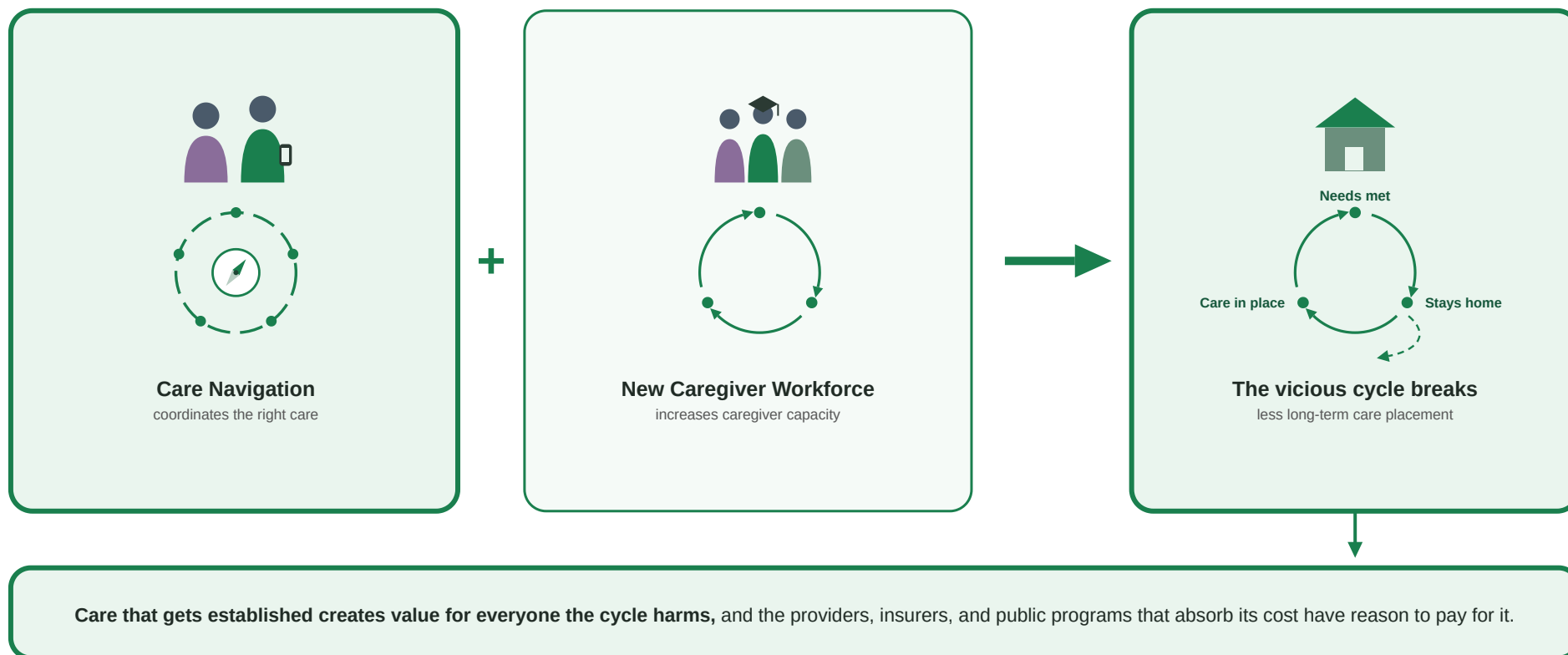
4. To Break the Cycle, You Must Navigate a Complex Eldercare Ecosystem



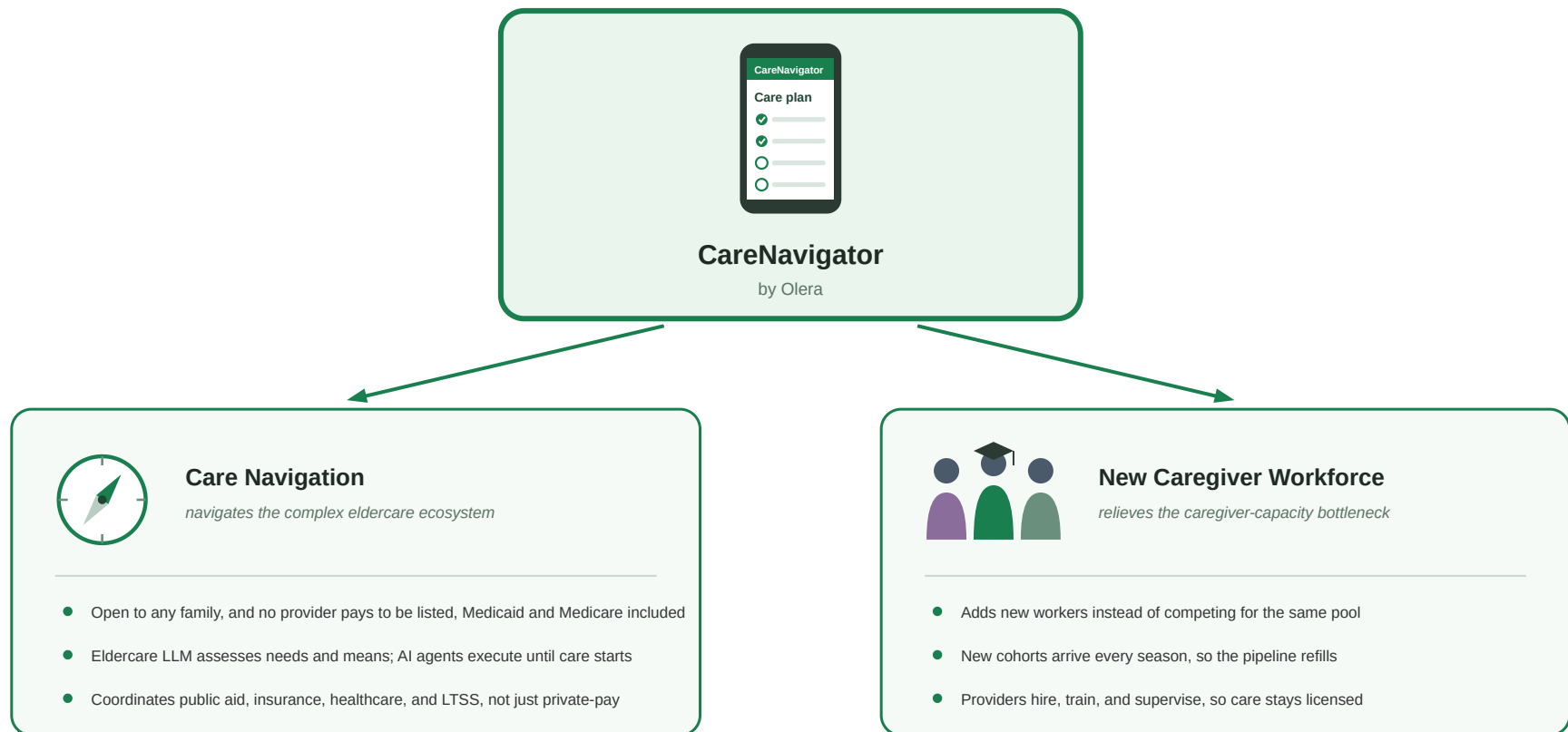
5. Navigation Alone Encounters a Major Caregiver-Capacity Bottleneck



6. Therefore, Navigation and New Capacity Together Break the Vicious Cycle

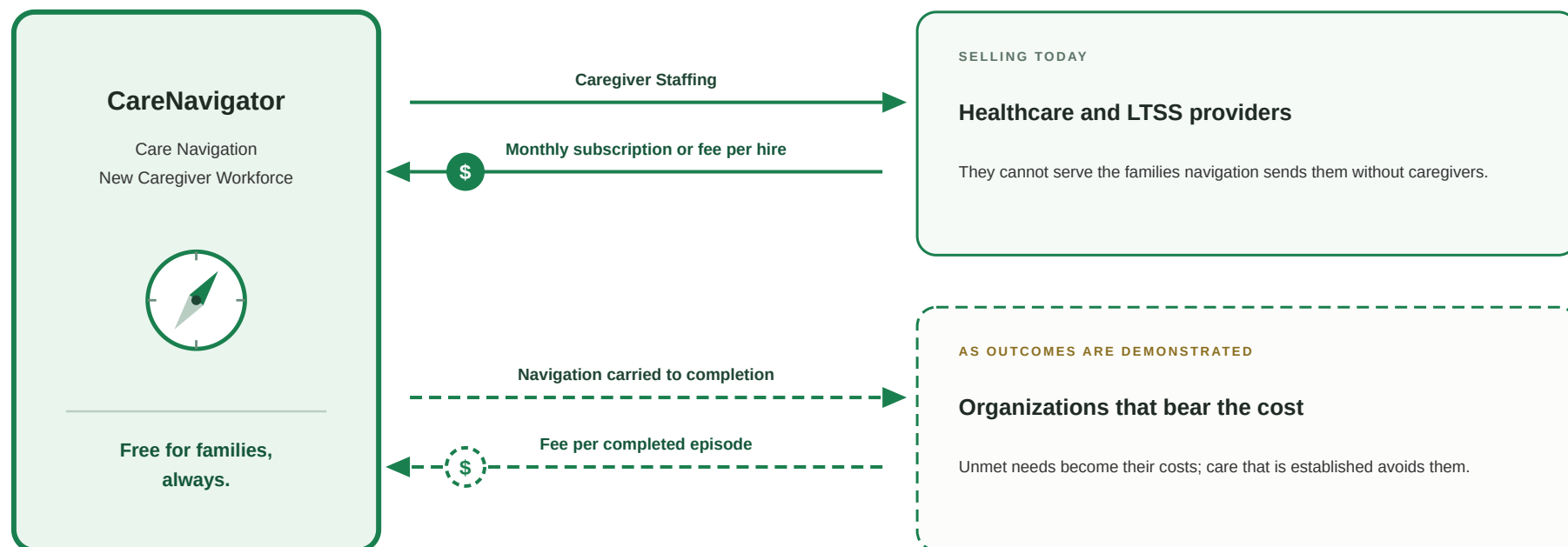


7. So, Olera Is Building CareNavigator to Do Both



Developed in Innovation: Key Innovation 1 (AI-assisted navigation), Key Innovation 2 (new caregiver workforce), Key Innovation 3 (aid, provider, and outcomes database).

8. But to Sustain Both Solutions, We Must Generate Revenue

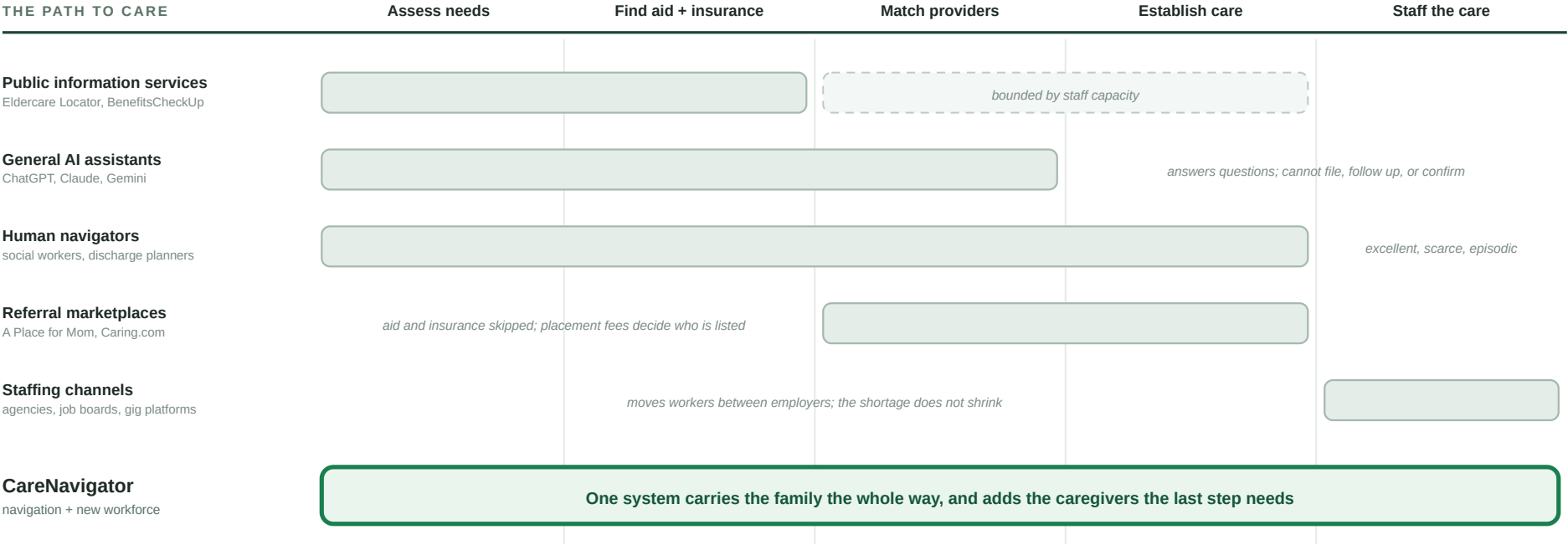


9. Business Model: Customers, Products, and Revenue Streams

Customer segment	Why they pay	Olera offering	Payment model	Illustrative economics
Beachhead: selling today				
Healthcare and LTSS providers	Staffing is the binding constraint. Unfilled shifts cap the clients they can take.	Caregiver Staffing. Olera recruits and vets; the provider hires and supervises.	Per hire or monthly subscription	~\$275/month or ~\$150 per hire ~25 providers, ~\$80K/year
Emerging: as preventive geriatric care outcomes are demonstrated				
Medicare Advantage plans	Avoidable utilization is their cost; retention and star ratings are their revenue.	Sponsored navigation episodes for members they identify, verified back	Per completed episode, then PMPM	~\$400 per episode ~120 episodes, ~\$50K/year
Medicaid managed care organizations	Institutional care costs far more than home care, and waivers go underused.	Sponsored navigation episodes, waiver-focused	Per completed episode	~\$500 per episode ~50 episodes, ~\$25K/year
Health systems and ACOs	Delayed discharges cost bed-days; readmissions carry penalties.	Discharge navigation episodes, with closure reported back	Per completed episode	~\$200 per episode ~80 episodes, ~\$15K/year
Self-insured employers	Working family caregivers lose time and productivity to eldercare crises.	Caregiver-support benefit for employees	Per employee per month	~\$2-4 per employee/month 1 contract, ~\$25K/year
Public aging agencies	They field the calls but lack casework capacity to follow through.	Contracted navigation behind their existing helplines	Public contract or grant	~\$25K/year per contract
Illustrative total, one mid-sized market (one county) at maturity, beyond the award				~\$220K per year

Assumptions, per-market economics, and the revenue build in Commercialization Plan, Section 9 (Revenue Streams). Beachhead price is shown as the two pilot-derived hypotheses Aim 3 tests.

10. But There Is Competition



The advantage is coverage, not features. Every alternative solves one stretch of the path and stops at the handoff that loses families.

11. And Related Academic Efforts

	WHAT THIS WORK HAS ESTABLISHED	WHAT REMAINS OPEN, AND WHAT WE DO ABOUT IT
Unmet need research NHATS and Medicare-linked cohorts	Unmet ADL and IADL need predicts emergency visits, readmission, and placement.	Whether that cascade can be interrupted outside a trial. We take this as settled and measure care established, not clinical endpoints.
Home-based support trials CAPABLE, Johns Hopkins	Function-focused home support reduced disability across randomized trials, with Medicaid savings well above program cost.	How a family outside a trial finds such support, funds it, and gets it started. CareNavigator is the front door these programs assume exists.
Digital caregiver tools and AI including our published work	Caregiver platforms can be built and measured as usable and accepted, four peer-reviewed studies of ours among them.	Whether AI agents can complete applications and follow a case to decision. Key Innovation 1 tests execution, not information.
Payer-funded navigation Medicare GUIDE model, 2024	Medicare now pays for care navigation, caregiver support, and respite in dementia.	Whether navigation beyond dementia is paid for the same way. The emerging revenue in Slides 8 and 9 rests on this answer.
Health-professions pipeline admissions requirements research	Admission to health-professions programs requires documented patient-care hours.	Whether that requirement converts into durable eldercare capacity. Key Innovation 2 tests recruitment, retention, and provider economics.
Much of the science is settled. What remains open is operational: whether execution and new capacity can be delivered where families actually live.		

12. And Hurdles to Adoption

THE PRIMARY HURDLE: LOCAL DENSITY AT A COST THE MARKET SUSTAINS

Families, providers, and caregivers must concentrate in one local market, and that market's revenue must cover the cost of bringing them together.

FAMILIES

Families must trust AI with care



Answers come from an expert-curated database, audited by domain experts.

Aim 1: acceptance validated under IRB

PROVIDERS

Providers are wary of platforms



Providers start free and pay only after the product has delivered value.

Aim 3: conversion and churn under real billing

WORKFORCE

New caregivers must be safe and welcome



Licensed providers hire, train, insure, and supervise them as employees.

Aim 2: the pathway measured end to end

EVERY MARKET

Each market must repeat affordably



Self-service portals and a documented market-entry playbook.

Aims 1 and 2: acquisition cost per market

Innovation

Slides 14 to 17

THE RESEARCH STRATEGY

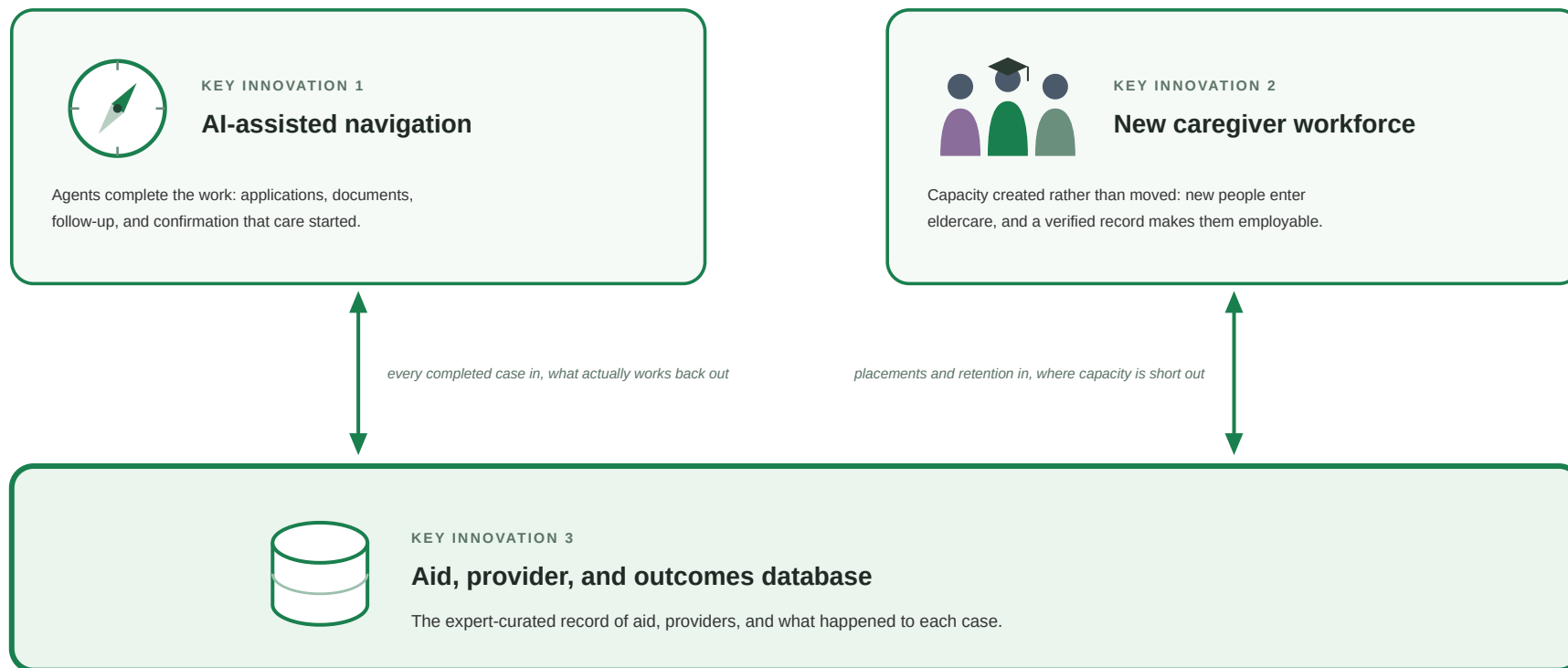
Significance

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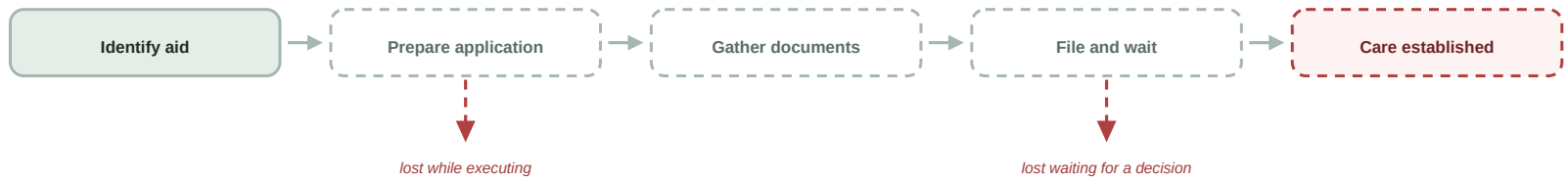
14. But We Have Three Key Innovations



15. Key Innovation 1: AI-Assisted Navigation

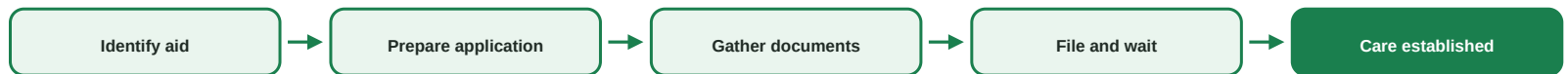
TODAY

tools inform; the family executes



WITH AI AGENTS

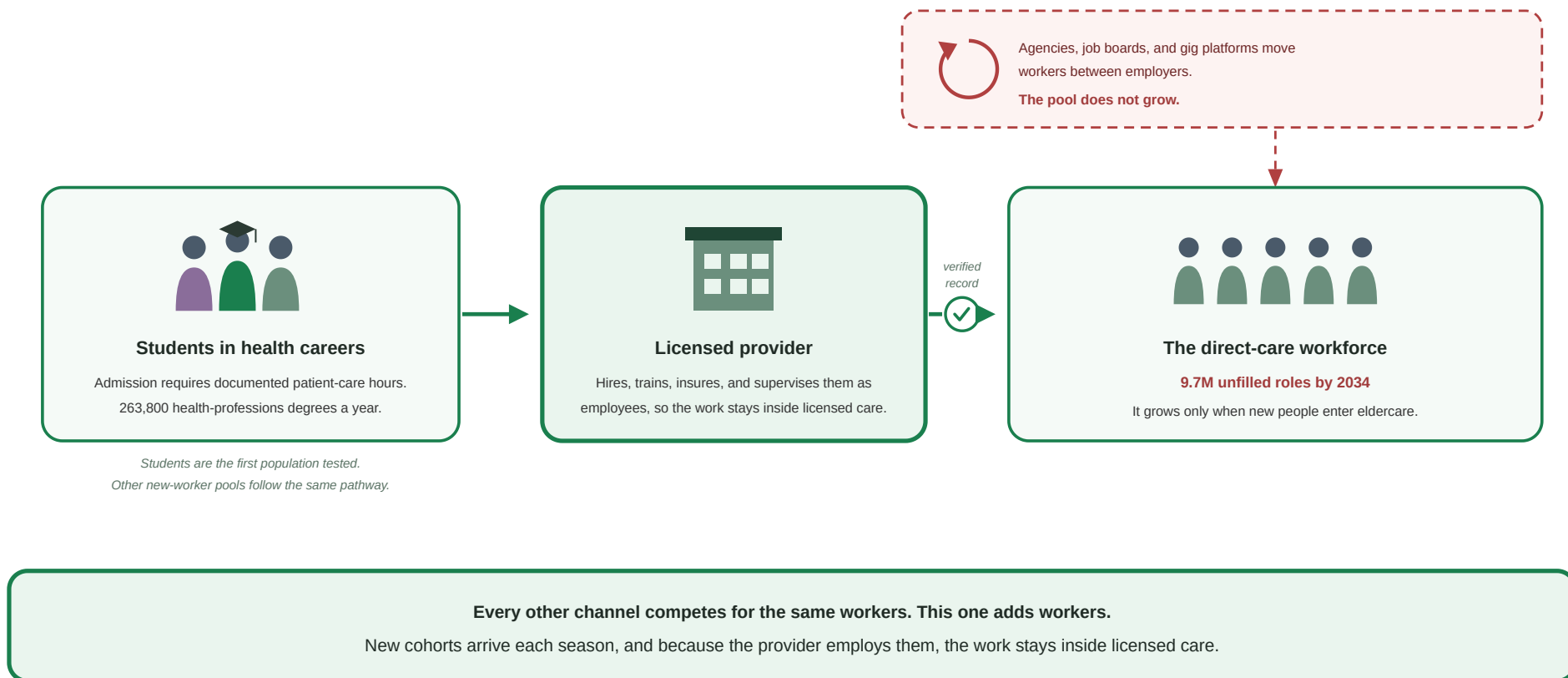
agents execute; the family decides



The unit of success changes from information delivered to care established.

The same families already arriving convert instead of falling out, and every completed case teaches Key Innovation 3.

16. Key Innovation 2: New Caregiver Workforce



17. Key Innovation 3: Aid, Provider, and Outcomes Database

PUBLISHED ON THE OPEN WEB

Public directories and program rules, which are often neither accurate nor current.

Available to anyone, including general-purpose AI

NEVER PUBLISHED

Local programs and provider realities, gathered by field work, expert curation, and direct outreach.

72,000+ records and 700+ guides, assembled in Phases I-IIIB

LEARNED ONLY FROM COMPLETED CASES

What programs actually decide, county by county, and what happened after.

Built by this project, case by case

what was needed

what qualified

where it stalled

whether capacity existed

what care started

A competitor who copies the features starts with an empty database.

Ours becomes a county-by-county record of unmet need and what resolved it, which no public dataset contains.

Approach

Slides 19 to 27

THE RESEARCH STRATEGY

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CRP Progress Report

19. So What Would Our Research Approach Actually Prove?

WHAT SLIDES 3 TO 17 CLAIM

Families reach established aid and care

AI agents execute at a quality a licensed professional would approve

The database learns with every completed case

We add new caregiver capacity to the system

Providers will pay for staffing

The economics hold across multiple market types

Established care means less avoidable utilization and placement

WHERE THE PROOF COMES FROM

AIM 1

Establish care reliably

AIM 2

Bring new caregivers into the workforce

AIM 3

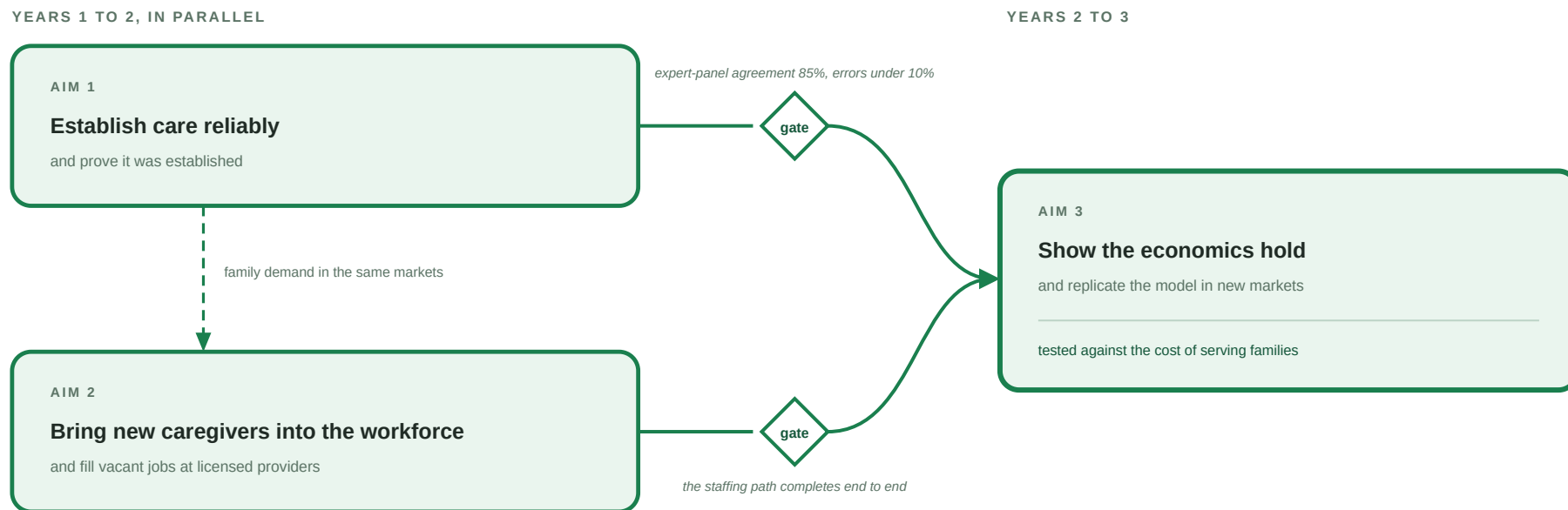
Show the economics hold

NOT AN AWARD ENDPOINT

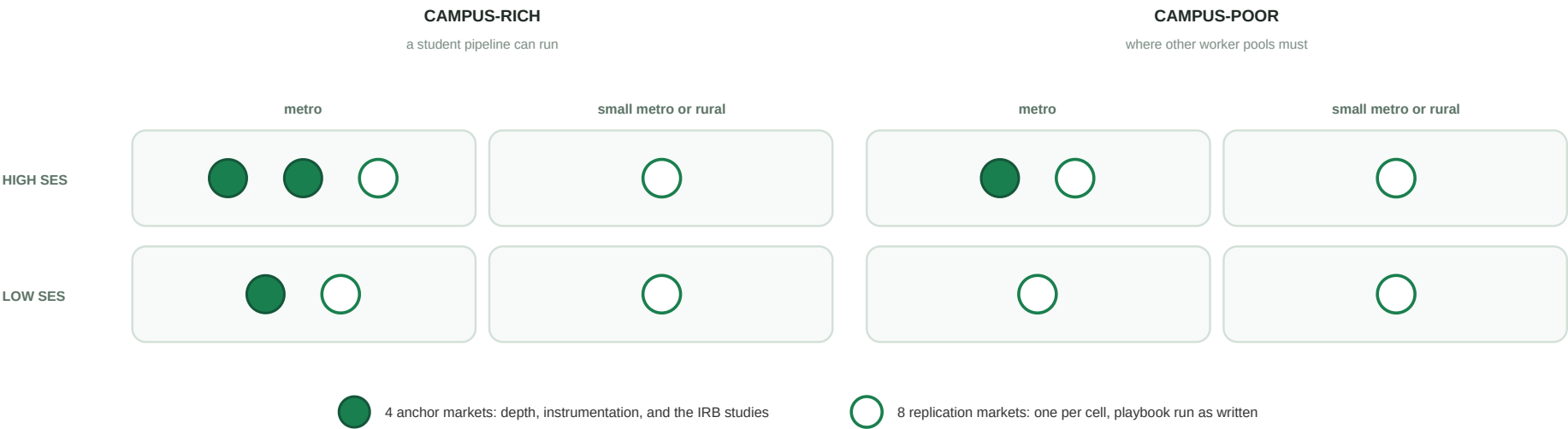
Supported by cited literature and an actuarial model



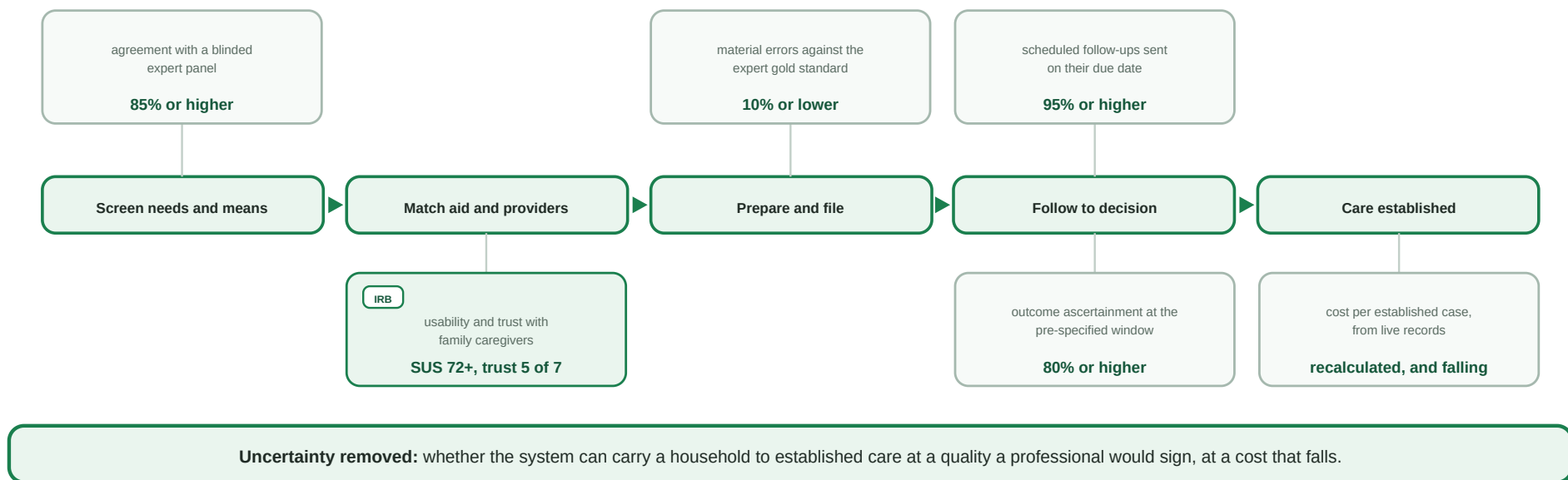
20. Three Aims, One Chain of Evidence



21. Where the Work Happens First, and Where It Replicates



22. Aim 1: Can Care Actually Get Established?



AIM 1 TASKS

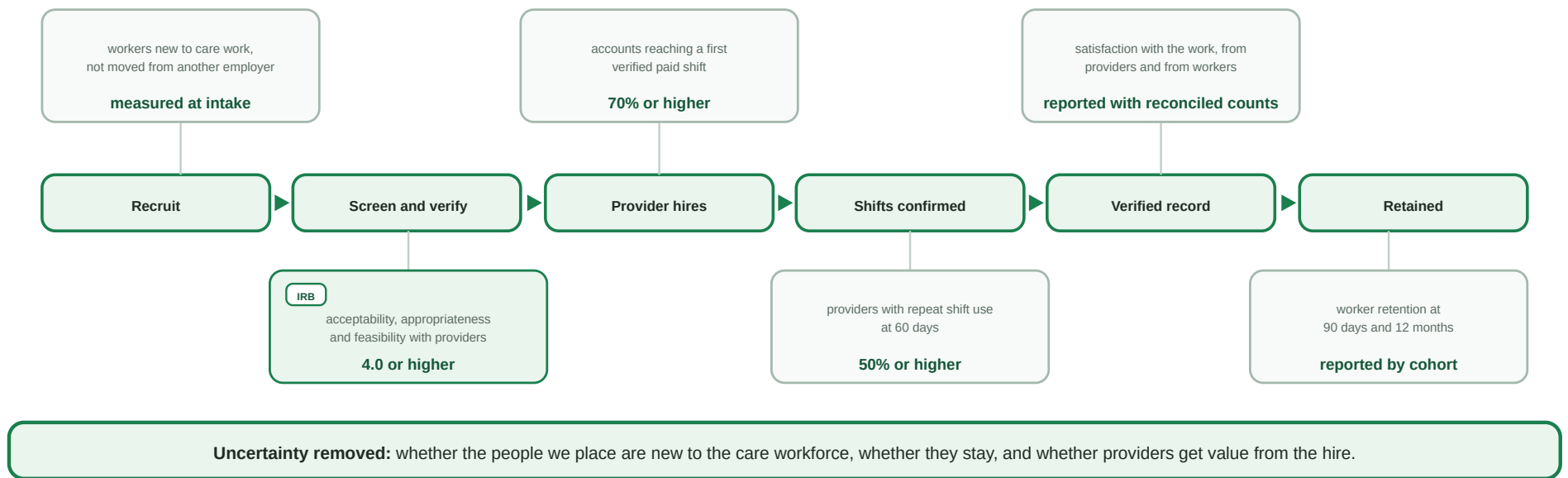
Task 1.1 Verify the execution and follow-up loops against blinded expert review.

Task 1.2 Validate with 25 family caregivers under IRB approval the usability, trust, task completion, and where the system loses people.

Task 1.3 Measure cost to acquire and cost to serve from live records.

Reported throughout: eligibility accuracy against the volume of executed cases, to test whether the database sharpens with use.

23. Aim 2: Can We Bring New Caregivers Into the Workforce?



AIM 2 TASKS

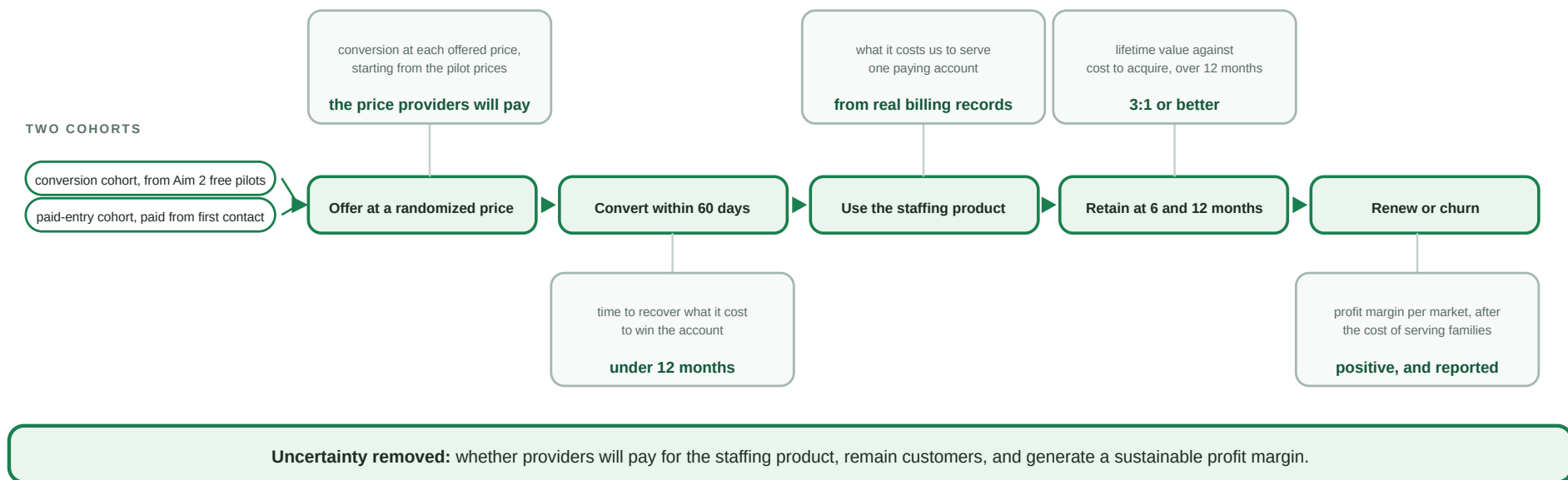
Task 2.1 Verify the staffing pipeline end to end.

Task 2.2 Validate with providers and workers the acceptability, appropriateness, and feasibility of the job.

Task 2.3 Measure cost per applicant, per hire, and per verified shift.

Throughout: providers pay Olera nothing for staffing in Aim 2. Paid conversion is tested in Aim 3.

24. Aim 3: Will Providers Pay, and Do the Economics Hold?



AIM 3 TASKS

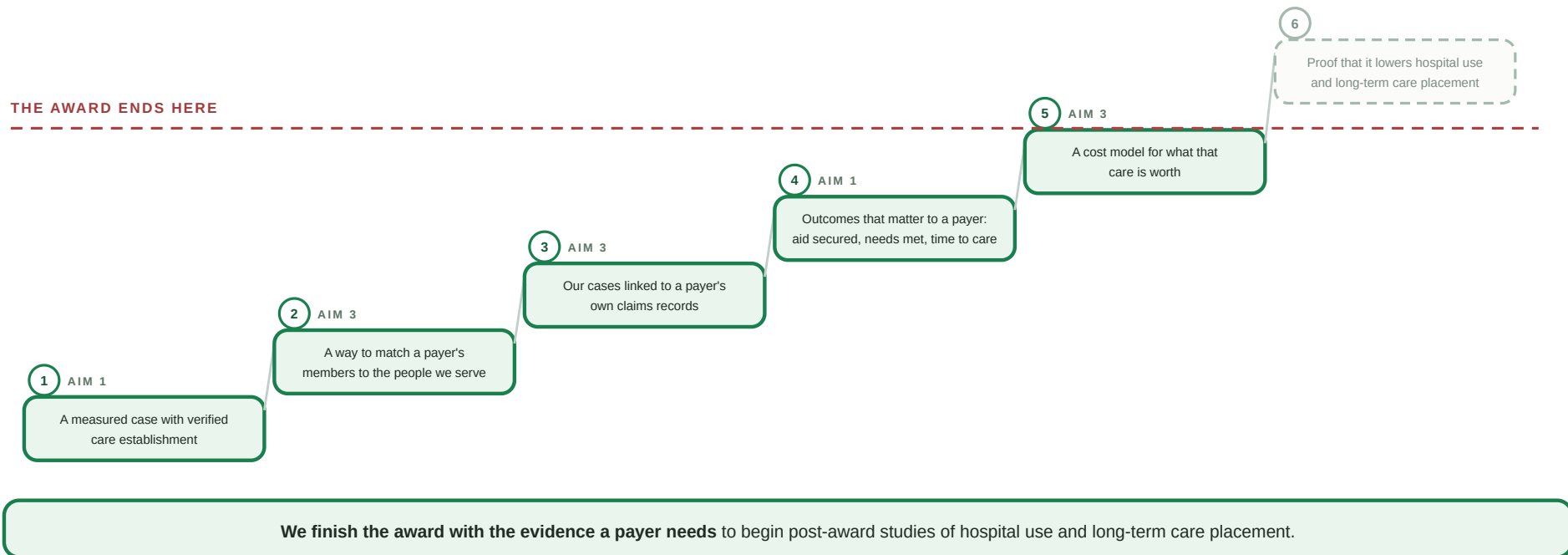
Task 3.1 Set pricing and packaging under real billing. Start from the pilot-derived prices, about \$275 a month or \$150 a hire, then find what providers will actually pay.

Task 3.2 Measure unit economics and retention: lifetime value against acquisition cost over 12 months, payback, churn, and margin.

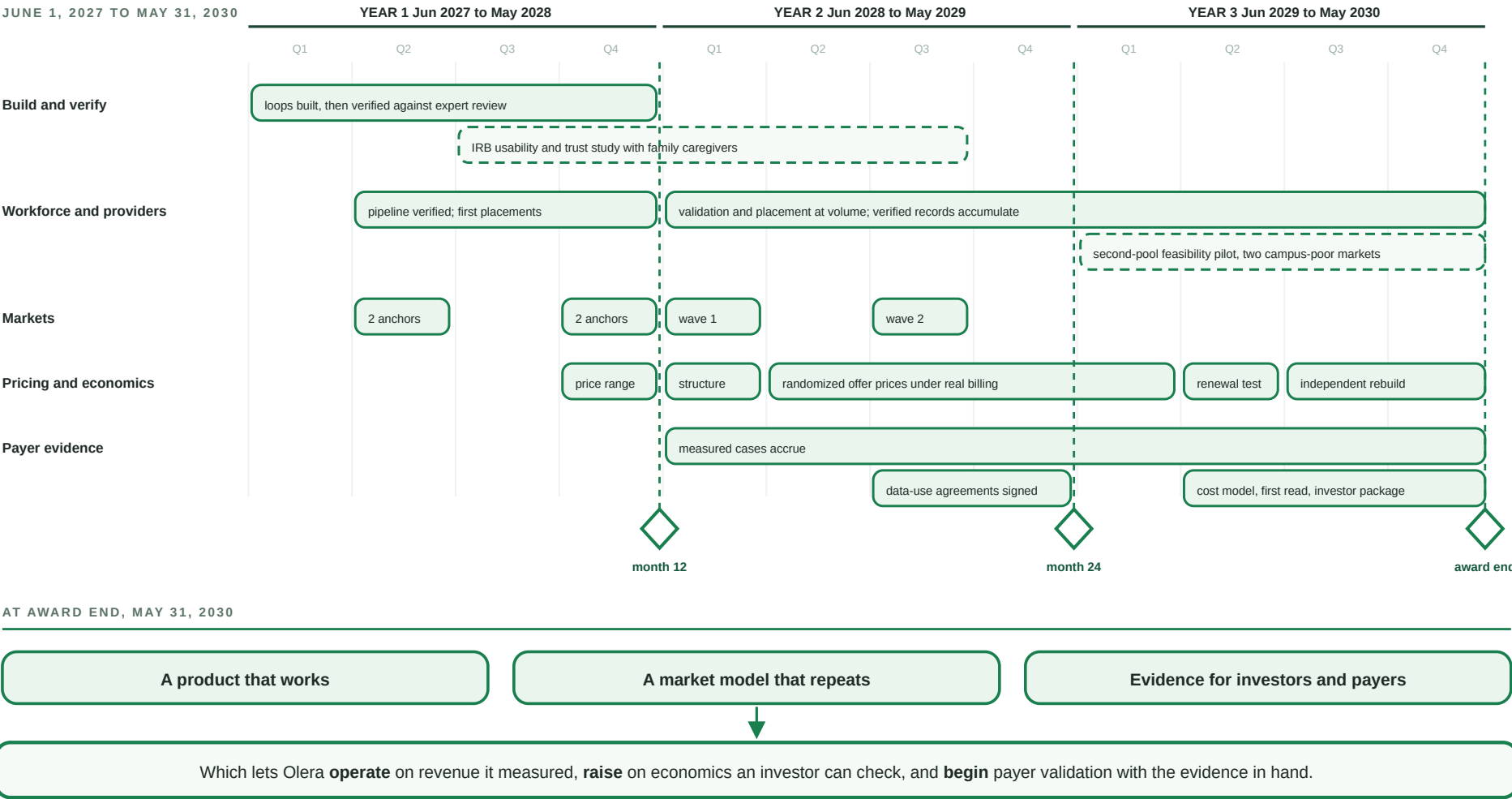
Task 3.3 Prepare the unit-economics package and the market-capture playbook for investor readiness, with an independent analyst rebuilding the economics from billing and cost records.

Task 3.4 Prepare the payer evidence package: care established, and the strongest defensible evidence on utilization and economic value, estimated where it is not directly measured.

25. While We Develop Staffing, We Build the Case for Future Payer Products



26. Three-Year Timeline and End Result



27. No FDA Pathway Stands in the Way

Does the product diagnose, treat, or make clinical decisions?



No. It is navigational and administrative software.

not a medical device

Does the project prospectively assign anyone to an intervention to measure a health outcome?



No. Health outcomes appear only as secondary, observational measures.

not an NIH-defined clinical trial

Does any activity involve human subjects?



Yes. Three bounded studies, each under IRB approval.

determinations obtained, not assumed

WHAT DOES GOVERN, AND WHERE IT IS ADDRESSED

- PrivacyHIPAA-aligned safeguards for family data
- Marketing and referral practiceno fee is charged for a referral, so no steering incentive exists
- Employment and supervisionlicensed providers employ, train, insure, and supervise every worker
- Payer linkagea data-use agreement and an IRB determination before any record is joined

No premarket authorization is required before commercialization. Nothing in the plan waits on a federal approval.

CRP Progress Report

Slide 29

THE RESEARCH STRATEGY

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29. What Two NIH Awards Already Bought

ALREADY RETIRED

TECHNICAL RISK

NIA Phase I to IIB

Could the data even be assembled?

We built a national database of more than 72,000 aid programs and providers in all fifty states, and an eldercare AI that runs on it. Adding that AI to the live product is underway.

USER RISK

NIA Phase I to IIB

Would families accept it?

Four peer-reviewed studies scored the platform 4.57 of 5 for usability and 5.83 of 7 for acceptance after four weeks of use. The AI version scored 5.73 of 7 in a study now being written up.

DEMAND RISK

Beyond the Phase II scope

Would families come, and at what cost?

Visits grew from about 50 a day in 2023 to more than 500 today, near 15,500 a month, from almost every county in the country. Search brings them, so reaching them costs us almost nothing.

SUPPLY RISK

I-Corps and company funds

Would providers show up?

We interviewed more than 200 owners and operators through NIH and NSF I-Corps, and over 700 providers have since claimed their listing for free. About 150 more claim one each month.

WORKFORCE RISK

Beyond the Phase II scope

Could we recruit caregivers, and would anyone pay for them?

A student caregiver pilot drew more than 900 applicants and placed over 20 into provider jobs. Franchisees of four national brands and one assisted-living community paid or agreed to pay.

NOT YET RETIRED

THE REMAINING RISK

Can one local market pay for itself, and can we repeat it?

Nine years of NIH work bought everything to the left. It did not buy this, because it cannot be bought. It has to be measured in real markets, with real money changing hands.

Aims 1, 2, and 3

Commercialization Plan

Sections as they stand in the Living Commercialization Plan. None yet developed.

- ☐ 31 1. Statement of Need
- ☐ 32 2. Value of the CRP Project, Expected Outcomes, and Impact
- ☐ 33 3. Company
- ☐ 34 4. SBIR/STTR Commercialization History
- ☐ 35 5. Market, Customer, and Competition
- ☐ 36 6. Intellectual Property Protection
- ☐ 37 7. Finance Plan
- ☐ 38 8. Product and Marketing Plan
- ☐ 39 9. Revenue Streams
- ☐ 40 11. Project Management Plan

Statement of Need

Not yet developed

THE COMMERCIALIZATION PLAN, BY SECTION

- 1
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Value of the CRP Project, Expected Outcomes, and Impact

Not yet developed

THE COMMERCIALIZATION PLAN, BY SECTION



Company

Not yet developed

THE COMMERCIALIZATION PLAN, BY SECTION

1

2

2

4

□

1

SBIR/STTR Commercialization History

Not yet developed

THE COMMERCIALIZATION PLAN, BY SECTION



Market, Customer, and Competition

Not yet developed

THE COMMERCIALIZATION PLAN, BY SECTION



Intellectual Property Protection

Not yet developed

THE COMMERCIALIZATION PLAN, BY SECTION



Finance Plan

Not yet developed

THE COMMERCIALIZATION PLAN, BY SECTION

- 1
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Product and Marketing Plan

Not yet developed

THE COMMERCIALIZATION PLAN, BY SECTION



Revenue Streams

Not yet developed

THE COMMERCIALIZATION PLAN, BY SECTION

- 1
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Project Management Plan

Not yet developed

THE COMMERCIALIZATION PLAN, BY SECTION

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